TRAUMA-04 | For therapists | Two separate age-based pathways

Mapping meaning and model-specific formulation

1 | Purpose, fit and boundaries

Purpose of this unit

Support identifying situation, thought and emotion in the person's words. For children, this aids adapted cognitive language; for adults, it supports identifying meaning and a stuck point to explore with the therapist.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

Do not argue with memory, assign responsibility for the event or demand positive thoughts. Assess current-danger thoughts against reality. This sheet is not an investigative or fact-finding tool.

Reusing existing material

Core forms may help distinguish thoughts from feelings but do not replace TF-CBT or CPT formulation. Unit 4 identifies a question to work on; it does not dictate conclusions.

A fictional example for discussion

Fictional example: Noam says asking for help means weakness. The therapist helps identify this as a meaning he assigns and formulate a question, without deciding his thoughts for him.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | Age-based pathways

CH / TE | Support within a TF-CBT pathway

CH/TE sheets distinguish situation, thought and feeling and formulate a question. Cognitive coping and further components are selected within TF-CBT; do not call every child's thought a CPT stuck point.

AD / OA | Support within a CPT pathway

AD/OA sheets bring personally worded meaning or a stuck point to CPT discussion. Do not copy an official worksheet here or infer that the belief has changed from these questions.

Parallel work with a suitable support person

Supporters can learn to hear the young person's thoughts without debate or correct answers. Supporter beliefs and guilt need separate work; do not use the child's sheet to process adult distress.

Using W and P sheets

Choose the age version and relevant questions; completing every sheet is not a condition for progress. W accompanies professional conversation. P records only pre-agreed follow-up. Unit number does not mean session number.

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

Do not affirm self-blame or dismiss it with a slogan. Explore meaning, context and knowledge available then. Sensitive details need not be written.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

CH chooses a simple situation the child agrees to explore; speaking is enough. Avoid leading event questions or turning conversation into detailed interrogation.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

TE uses the teenager's words and separates interpretation from information. Changed wording does not prove agreement; do not share personal thoughts without the appropriate agreement.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

AD explores a stuck point within CPT, not as a label for every emotion or difficulty. Explore meaning while separating knowledge at the event from hindsight.

OA | Older adults

OA respects life experience and complexity of loss, roles and relationships. A longstanding belief is not wrong because it is longstanding; choose access and pace with the person.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

P records support and learning agreed in treatment. It sets no exposure, trauma narrative, dosage or component sequence. Without a plan, bring questions to the session. CH/TE sheets do not replace separate supporter work; AD/OA sheets are not official CPT materials.

At the next session

Check improved distinction between current events, thoughts and feelings. Changed wording need not mean changed belief. Choose further model-specific work together.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Proposed: TF-CBT for children/teens; CPT for adults/older adults. Full manuals remain unmapped. Age, rights, supporter materials and separate-guide needs await review. Original support drafts, not an approved protocol.

www.nctsn.org/interventions/trauma-focused-cognitive-behavioral-therapy
tfcbt.org/about/
www.ptsd.va.gov/professional/treat/txessentials/cpt_for_ptsd_pro.asp

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